AR-04 Adult COPD / Asthma

CLAIBORNE COUNTY EMS | MEDICAL-DIRECTOR APPROVED | SOURCE PDF | OCTOBER 17, 2026

Scope: Adult respiratory distress from suspected asthma, COPD, reactive airway disease, bronchospasm, or stridor. Differentiate lower-airway wheeze from upper-airway stridor and other causes of distress.

Field flow

1. Assess	Work of breathing, speech, air movement, SpO2, lung sounds, mental status, fatigue, and competing causes.
2. Support	Position upright, oxygen target, albuterol with ipratropium for moderate/severe bronchospasm.
3. NIPPV	AEMT/Paramedic: use early only if cooperative and hemodynamically stable.
4. Escalate	Severe/not improving: epinephrine, steroid, magnesium, BVM/advanced-airway preparation, reassessment, transport.

Provider-level actions

Level	Approved actions
EMT	Oxygen, comfort position, authorized albuterol, assisted ventilation if tiring, and immediate epinephrine for anaphylaxis within scope.
AEMT	All EMT care plus NIPPV for an appropriate patient, vascular access, repeated albuterol/ipratropium, nebulized epinephrine for stridor, and authorized IM epinephrine.
Paramedic	All prior care plus cardiac/EtCO2 monitoring when indicated, steroid/magnesium, advanced airway, and evaluation of competing diagnoses.

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SAFETY, MEDICATION, AND LINKED-PATHWAY REFERENCE

Safety and escalation

• Target SpO2 93-95% for asthma or undifferentiated hypoxemia. For known COPD with hypercapnia risk, usual lower saturation, or an individualized plan, target 88-92%; never withhold oxygen from critical hypoxemia.

• Do not persist with NIPPV for hypotension, worsening mental status, emesis, inability to protect airway, intolerance, or deteriorating ventilation.

• A silent chest with distress is a pre-arrest finding; do not delay BVM and airway preparation for repeated nebulizers.

• Medication administration follows current Claiborne authorization and standing orders.

Medication reference

Medication	Approved dose / safeguard
Albuterol	2.5-5 mg nebulized; repeat as needed x3 and continue when indicated
Ipratropium	0.5 mg nebulized with initial albuterol for moderate/severe bronchospasm
Epinephrine 1 mg/mL	0.3-0.5 mg IM for anaphylaxis, severe upper-airway edema, or life-threatening asthma
Nebulized epinephrine	1 mg of 1 mg/mL in 2 mL normal saline; may repeat once for stridor/upper-airway edema
Methylprednisolone	125 mg IV/IO/IM
Magnesium sulfate	1-2 g IV over 15-30 min for severe nonresponsive bronchospasm

Linked pathways

AR-01 Adult Airway; AR-03 Drug-Assisted Airway; AM-01 Allergic Reaction / Anaphylaxis; AM-05 Hypotension / Shock; AM-07 Crashing Medical Patient.

Source control

• This controlled direct source PDF replaces the prior source file and matches approved Claiborne EMS app content released October 17, 2026.

• Use current Claiborne standing orders, formulary, equipment authorization, and Medical Control direction whenever they govern a case-specific decision.